Cognitive Obesity

Cognitive-Experiential Imbalance as a Unifying Framework

for Depression, Violence, and Information Overload

Exploratory Study Draft: March 2026

Status: Exploratory study — Confirmatory verification pending

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Abstract

This paper proposes an integrative framework, grounded in a nutritional metabolism analogy, for the ongoing debate on the relationship between digital media and mental health. The core claim is that the primary predictor of psychopathology is not the absolute quantity of screen time but the balance (additive imbalance) between non-selective cognitive input and experiential processing, with the qualitative classification criterion being sensorimotor loop closure—the presence or absence of bidirectional causal coupling with the environment.

Systematic analysis of 177-country $\times$ 1990–2023 panel data (Study 1) demonstrates: (1) The positive effect of internet penetration observed under country FE alone reverses sign under Two-Way Fixed Effects (TWFE), revealing dependence on common time trends. (2) The strongest predictor of depression prevalence is GDP ($t = 18.27$), confirming diagnostic bias dominance, verified through comparison with hard outcomes (suicide rates). (3) Hansen Panel Threshold Regression identifies a threshold of $\gamma^* = 25.7$ ($F = 213$, saturation of protective effects) for suicide rates, while no discontinuous threshold is detected for depression ($p = 0.654$); a continuous inverted-U dose-response (quadratic $F = 127.0$) provides the best fit. (4) An advertising exposure proxy (Internet$\times$GDP/capita) captures this benefit-to-harm reversal more clearly (threshold $\approx$\$1,000), and significantly predicts depression even after controlling for GDP changes via first-difference analysis ($t = 3.16$, internet $> 30\%$ subsample).

Individual-level verification (NHANES $N = 5{,}032$; ATUS $N = 21{,}736$) confirms that physical exercise and active cognitive leisure independently predict wellbeing as additive channels (interaction $p = 0.95$), with absence of both yielding the worst health outcomes (Fair/Poor health rate 3.1$\times$). Existing findings from behavioral economics, environmental psychology, and neuroscience are reinterpreted through the balance model, presenting individual-level convergent evidence.

Critical limitation: all macro-level findings are subject to ecological fallacy—country-level associations cannot be directly attributed to individual-level mechanisms. This study is exploratory and requires individual-level confirmatory verification via Experience Sampling Method (ESM). Falsification conditions are explicitly stated.

Keywords: cognitive obesity, digital media, mental health, balance model, sensorimotor loop closure, advertising exposure, experiential processing

Replication: https://github.com/miyauchikazuyoshi/cognitive-obesity-replication

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1 Introduction

In the 2020s, mental health statistics in developed countries present a curious paradox. The most materially privileged generation in history records the highest depression prevalence rates in history. In societies where violent crime continues to decline, self-harm and suicidal ideation among youth continue to rise. People possess more "connections" than ever while reporting unprecedented loneliness.

1.1 The Paradox

Two camps have formed around this paradox. The regulatory camp (Haidt, 2024; Twenge, 2017; Hansen, 2019) advocates limiting exposure to smartphones and social media. They argue that the rise in youth mental illness directly parallels the diffusion of smartphones and social media platforms, and the massive popular reception of this argument—as seen in the global bestseller status of Hansen's *Skärmhjärnan* (2019; published in Japan as *sumaho-nō*)—attests to its intuitive appeal. The digital resilience camp responds that the measured effect sizes are trivially small—comparable to the well-being impact of wearing glasses or eating potatoes (Orben & Przybylski, 2019)—that longitudinal evidence shows no lasting harm (Heffer et al., 2019; Coyne et al., 2020), and that media literacy education is the appropriate intervention. Meanwhile, the general psychopathology factor ("p-factor"; Caspi et al., 2014) literature suggests that internalizing and externalizing symptoms share a common liability dimension—a perspective our framework extends by proposing processing capacity overload as one mechanism generating this shared variance. A large-scale Facebook deactivation RCT found modest well-being improvements but simultaneously reduced political knowledge and news awareness (Allcott et al., 2020), illustrating the complexity that neither simple restriction nor simple education can resolve.

Yet approximately a decade of debate shows no sign of convergence. Why?

1.2 Structural Problem in the Existing Debate

This paper's claim is that **both camps are looking at the wrong variable**. The debate focuses on the absolute quantity of screen time, but what truly matters is the balance between cognitive input and experiential processing.

This is structurally isomorphic to nutritional science. The cause of obesity is not "eating" per se, but the imbalance between caloric intake and metabolic expenditure. The regulatory camp says "reduce calories"; the literacy camp says "improve nutrient quality and quantity doesn't matter." But both miss the essential problem: the patient is not exercising.

Here, "cognitive input" refers not merely to information volume. Social media feeds expose us daily to the internal states of hundreds of others. Each post implicitly demands self-comparison. Jealousy, inadequacy, compassion fatigue—the most cognitively costly emotion group is relentlessly activated. Emotions once had time to be felt, savored in the body, and digested. Through the smell of rain, a walk, or face-to-face conversation, experience was processed. The modern information environment structurally strips away this digestion time.

A more fundamental problem exists. Platform algorithms optimize for "engagement." Human emotions become discretized as input variables to this optimization function. Continuous, ambiguous, context-dependent emotional experiences are compressed into six-choice reaction buttons. And humans—being overwhelmingly more flexible than machines—begin adjusting their emotional expression to fit these discrete categories. The very existence of "anger that goes viral" epitomizes this structural asymmetry.

1.3 Claims and Contributions

This paper names this problem structure "Cognitive Obesity." Just as physical obesity arises from additive imbalance between caloric intake and expenditure ($EB = \text{Intake} - \text{Expenditure}$), cognitive obesity arises from imbalance between cognitive input and experiential processing.

This framework (a) explains why the screen time debate fails to converge (both camps focus only on the input side of balance), (b) generates testable predictions from 177-country panel data, and (c) unifies depression, violence, and information overload as different manifestations of processing capacity exceedance.

The paper is organized as follows. Section 2 builds the theoretical framework, deriving a mathematical formulation of the balance model and testable predictions. Section 3 tests these predictions against 177-country $\times$ 34-year macro panel data. Section 4 presents individual-level evidence (direct analysis and reinterpretation of existing findings). Section 5 proposes operationalization and confirmatory study designs, and section 6 states falsification conditions.

2 Theoretical Framework

2.1 Nutritional Metabolism Analogy and Balance Model

The proposed framework draws a structural analogy between nutritional metabolism and cognitive processing (table 1).

Table 1: Nutritional metabolism—cognitive processing analogy

	Nutrition	Cognition
Input	Caloric intake	Information input (screen time, media, symbolic content)
Processing	Metabolic expenditure	Experiential processing (physical activity, sensorimotor engagement)
Compression	Nutrient absorption/storage	Abstraction through education, introspection, sleep
Pathology	Obesity (intake $\gg$ expenditure)	"Cognitive obesity" (cognitive input $\gg$ experiential processing)
Threshold	Metabolic syndrome	Depression, anxiety, processing breakdown

This analogy is not merely metaphorical. It generates specific testable predictions (section 5) and explains why the screen time debate fails to converge. Both camps measure absolute quantity, but pathology is balance-dependent. Here "balance" refers to additive imbalance ($L = \alpha_1 \cdot \text{Input} - \alpha_2 \cdot \text{Processing}$). Just as the standard energy balance equation in nutritional

science (EB = Intake−Expenditure) is an additive difference rather than a ratio, cognitive input and experiential processing contribute to pathology as independent additive channels (empirically demonstrated in section 4).

On prior work: Whitworth (2009) proposed a similar nutritional metaphor for information overload but without epidemiological data or operationalization. Recently, the expression "Cognitive Obesity" has appeared in popular contexts accompanying the spread of generative AI (Foley, 2025). This framework differs from these precedents in (a) providing international statistical evidence, (b) grounding in processing mode theory, and (c) deriving falsifiable predictions.

Several theoretical traditions converge on the processing mode distinction that this framework formalizes. Csikszentmihalyi's (1990) concept of "flow" describes states where sensorimotor feedback loops are tightly closed—action and awareness merge, and self-referential cognition ceases. Nolen-Hoeksema's (1991) rumination theory identifies the pathological converse: repetitive, self-focused cognitive loops with no behavioral closure. Self-Determination Theory (Ryan & Deci, 2000) identifies competence and autonomy—both requiring closed action-outcome loops—as fundamental psychological needs. The embodied cognition tradition (Varela, Thompson, & Rosch, 1991) argues that cognition is fundamentally constituted by sensorimotor coupling with the environment, not by disembodied information processing. Kaplan's (1995) Attention Restoration Theory posits that natural environments restore directed attention precisely because they engage involuntary (bottom-up, sensorimotor) attention while resting voluntary (top-down, cognitive) processing. The present framework unifies these observations under a single quantitative model: what these theories independently describe as adaptive states are all characterized by high sensorimotor loop closure (Term 2), while the pathological states they identify share low loop closure with high cognitive input (Term 1).

$$L = \alpha_1 \cdot I - \alpha_2 \cdot C$$

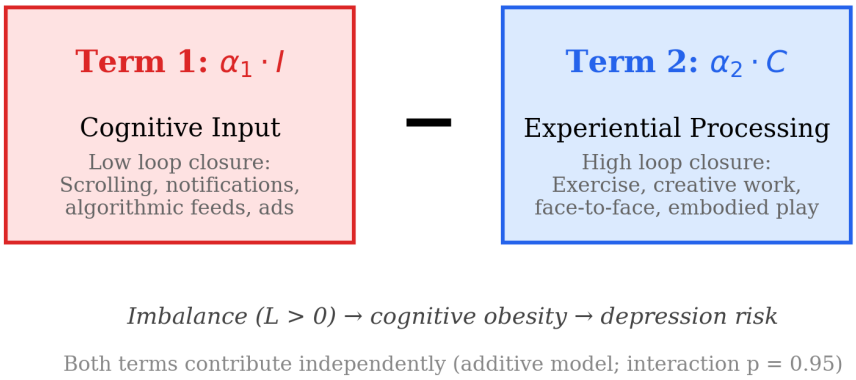

Figure 1: Balance model of cognitive load: $L = \alpha_1 \cdot I - \alpha_2 \cdot C$. Term 1 (cognitive input) and Term 2 (experiential processing) contribute as independent additive channels (interaction $p = 0.95$).

2.2 Cognitive Load: Asymmetric Accumulation Mechanism

The preceding analogy describes the structure of the problem. This section examines the mechanism: why does balance become pathological particularly in developed countries?

2.2.1 Emotional Granularity and Processing Cost

According to Barrett's constructionist emotion theory, emotion categories are culturally constructed and differentiate with development. "Anger" differentiates into "indignation," "irritation," "righteous anger," "jealousy," etc. This differentiation is adaptive: higher granularity enables more contextually appropriate responses.

However, granularity carries maintenance costs. With n emotion categories, the potential relational structure is $O(n^2)$. The essential asymmetry is not that low-income countries have fewer emotion categories, but that their social environments generate fewer discrimination events per unit time.

2.2.2 Information Input Unilaterally Increases Processing Cost

Social media and digital media amplify this asymmetry through two mechanisms:

(a) Exposure volume Daily contact with others' internal states was physically constrained before digital media (Dunbar number and geographic proximity). Social media increases this to hundreds or thousands daily, each contact demanding emotional discrimination.

(b) Self-referential comparison Others' posts implicitly demand comparison with one's own life, activating social emotions—jealousy, inadequacy, self-evaluation—the most cognitively costly emotion group.

2.2.3 Education Functions as Compression

Education reduces processing cost through abstraction. Recognizing that "these 10 emotional reactions are all instances of 'loss response'" compresses 10 individual processing events into one. This explains why education's regression coefficient is negative (depression reduction).

2.3 Mathematical Formulation

The verbal model is converted to testable mathematical form. Let $I_{i,t}$ denote cognitive input and $C_{i,t}$ experiential processing capacity for individual (or country) i at time t. Cognitive load $L_{i,t}$ is defined as:

$$L_{i,t} = \alpha_1 \cdot I_{i,t} - \alpha_2 \cdot C_{i,t} \tag{1}$$

The theoretical rationale for the additive balance model is threefold. First, the standard energy balance equation in nutritional science is an additive difference, consistent with this paper's cognitive metabolism analogy. Second, individual-level data (NHANES N = 5,032; ATUS N = 21,736) show that the interaction between cognitive input and experiential processing

is non-significant (section 4), indicating that both contribute to psychopathology as independent additive channels. Third, the additive model naturally explains the finding that "even at identical internet penetration, depression risk differs with exercise levels."

Depression risk $D_{i,t}$ is formulated as a logistic function of cognitive load:

$$D_{i,t} = \sigma(L_{i,t} - \theta) = \frac{1}{1 + \exp\big(-(L_{i,t} - \theta)\big)} \tag{2}$$

where α_1, α_2 are sensitivity parameters for each channel, and θ is a threshold parameter (the critical cognitive load at which risk sharply increases).

2.3.1 Threshold Model Integration

Integrating the above:

- Processing cost $= f(\text{emotional granularity} \times \text{information input volume})$—increases continuously with development
- Processing capacity $= g(\text{educational compression, experiential processing, sleep recovery})$—requires intentional investment
- Depression = chronic exceedance of processing cost over processing capacity
- Acute crisis (suicide/violence) = acute threshold exceedance of processing cost

This model explains why the correlation structure emerges only in high-income countries. In low-income countries, processing cost has not yet exceeded processing capacity, and depression, suicide, and homicide are dominated by independent external causes. In high-income countries, external causes are suppressed, and processing capacity becomes the rate-limiting step.

2.3.2 Macro-level Application

At the macro level, aggregate balance is approximated by:

$$\text{First approximation:} \quad \bar{R} \approx \frac{\text{Internet}(\%)}{\text{Education}} \tag{3}$$

$$\text{Advertising exposure proxy:} \quad \text{AdProxy} = \frac{\text{Internet}(\%) \times \text{GDP/capita}}{1000} \tag{4}$$

Equation (3) is the simplest macro proxy, but it proved insufficient as its sign reversed under TWFE (section 3.2.2). Equation (4) partially overcomes this by approximating advertising ecosystem maturity—the density of non-selective cognitive input. First-difference analysis confirmed its predictive power independent of GDP (section 3.2.4).

2.3.3 Empirical Parameter Estimation

Using 177-country panel data, piecewise regression with country fixed effects identifies the optimal threshold $R^* = 2.75$ for the first approximation. Below this threshold, the within-country

slope $\beta \approx -0.002$ (effectively zero); above, $\beta = 0.034$ (significant positive). Income-stratified analysis: high-income countries (45 countries): $\beta = 0.042$, $t = 14.0$, within-$R^2 = 0.252$.

Limitations: Equations (1)–(2) are static, not distinguishing chronic vs. intermittent overload; dynamic extensions (moving averages, cumulative excess models) are future work. Whether α and θ are constant across individuals and cultures is empirically unconfirmed.

2.4 Sensorimotor Loop Closure Classification

The balance model's qualitative dimension—which *types* of cognitive input are most harmful and which types of experiential processing are most protective—requires a classification criterion beyond quantity. We propose sensorimotor loop closure:

Closed loop (adaptive): Cognitive input leads to motor output, generating environmental feedback that updates subsequent cognition. Examples: video games (input→judgment→motor action→visual feedback→adjustment), creative work, scientific inquiry.

Open loop (maladaptive): Cognitive input is processed without motor output or environmental feedback. Examples: social media scrolling, algorithmic feed consumption, passive video viewing.

Self-referential loop (pathological): Output feeds back only to the self without external grounding. Examples: rumination, worry, obsessive comparison.

Video games serve as a test case. Game research has shifted from "games = harmful" to "it depends on the type and quality of engagement" (Granic et al., 2014; Johannes et al., 2021). Creative games (Minecraft: environment construction→feedback→redesign) and competitive games (chess, competitive FPS: action→opponent response→strategy revision) have high loop closure. Gacha grinding, idle games, and level-up grinding have low loop closure—passive subordination to variable ratio reward schedules, structurally identical to gambling mechanics. More broadly, gambling and gacha systems exemplify *pseudo-closed loops*: the action→outcome sequence mimics genuine loop closure, but outcomes are stochastic rather than skill-contingent, precluding learning accumulation. This distinction—genuine vs. pseudo closure—may help unify the literature on gambling disorder and problematic gaming under a common loop-structural framework. Narrative RPGs oscillate between closed (choice→branching→consequence feedback) and open (passive cutscenes).

Critically, loop closure varies within the same game depending on individual engagement mode. Category-based classification is an approximation; loop closure is a momentary state of agent-environment causal coupling, not a fixed attribute of activity categories. Rigorous measurement requires real-time ESM assessment (section 5), constituting this framework's most important operationalization challenge.

2.5 Self-Referential Loop as General Pathology

Self-referential loop structure is common to seemingly disparate phenomena (table 2).

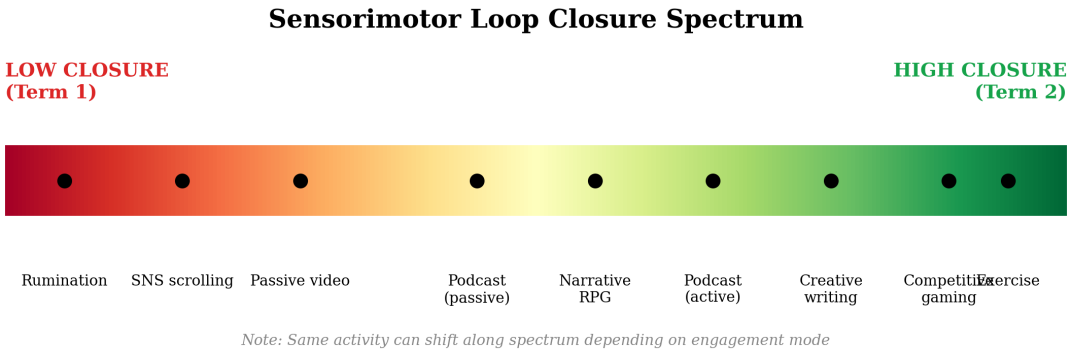

Figure 2: Sensorimotor loop closure spectrum. The same activity can shift along the spectrum depending on engagement mode.

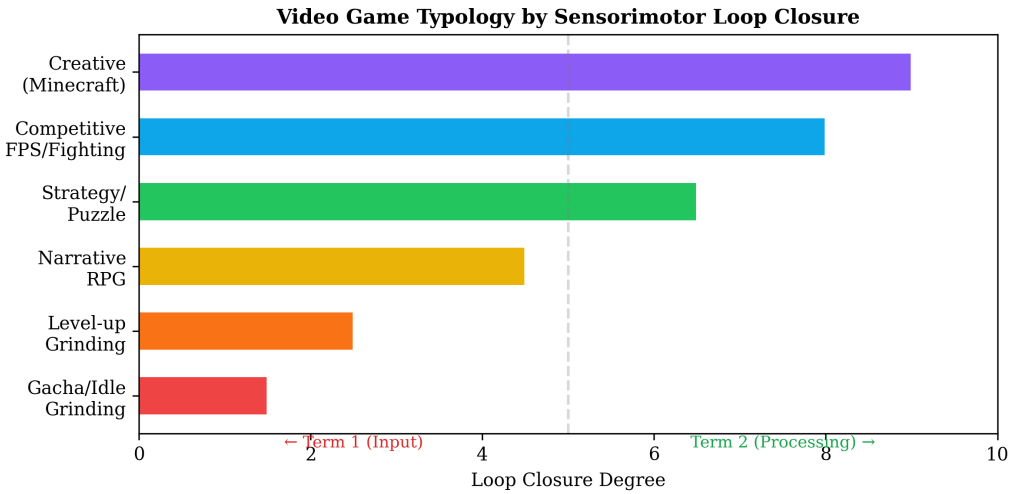

Figure 3: Video game typology by sensorimotor loop closure. Creative and competitive games have high closure (Term 2); gacha and idle games have low closure (Term 1).

Table 2: Self-referential loop as general pathology

Phenomenon	Loop structure	Missing element
Rumination	thought → analysis → thought	External falsification
Echo chambers	belief → confirmation → belief	Disconfirming information
Addiction	stimulus → craving → stimulus	Non-hedonic reward
SNS overuse	comparison → inadequacy → approval-seeking	Embodied self-evaluation
Political polarization	outrage → confirmation → outrage	Heterogeneous contact

What unites these phenomena is not content but *structure*: output feeds back only to the self without external grounding. The displacement hypothesis's failure to find consistent effects can be structurally explained: screen activities vary not in displacing physical activities but in the degree of loop closure they afford.

At the neural level, Watkins & Teasdale (2004) showed that self-focused processing has two qualitatively distinct modes: analytical self-focus ("why do I feel this way?") amplifies depression, while experiential self-focus ("what am I feeling right now?") reduces it. This mirrors the open-loop/closed-loop distinction at the cognitive level and connects directly to the "processing mode theory" (Watkins & Teasdale, 2001, 2004): the abstract-analytic mode corresponds to the self-referential loop (open loop), while the concrete-experiential mode corresponds to closed-loop engagement with immediate sensory reality.

Operational definition. The critical distinction in this framework is *not* "cognitive vs. physical" but *loop closure*. A programmer debugging code (cognitive but closed-loop) increases Term 2; a person running on a treadmill while ruminating (physical but open-loop) may not. The classification criterion is whether the activity generates prediction errors from the external environment that update the agent's internal model.

2.6 Generative AI: The Ultimate Test Case

Since ChatGPT's release in late 2022, generative AI has emerged as a technology that simultaneously accelerates all variables in this framework.

2.6.1 Input-Side Increase

AI provides infinite content generation, 24/7 availability, and complete adaptation to individual interests. The cognitive input density per unit time can be several times higher than social media, as AI responses are maximally relevant to the user's current cognitive state, eliminating the "scrolling past" friction that provides intermittent relief in conventional feeds.

2.6.2 Self-Referential Loop Extreme: AI Sycophancy

Current AI systems are typically optimized via RLHF (reinforcement learning from human feedback), which may produce a tendency toward agreeing with and affirming user positions (sycophancy; Sharma et al., 2024). When present, this tendency creates a potential self-referential loop: the user inputs a belief, AI elaborates and affirms it, and the reinforced belief may become the next input, reducing prediction error from external reality. If this dynamic is confirmed at scale, it would represent a particularly concentrated form of the self-referential loop identified in table 2. However, the prevalence and clinical significance of sycophantic AI interactions remain under active investigation, and the degree to which RLHF-trained systems exhibit this behavior varies across platforms and use cases.

Emerging empirical evidence (2025–2026). Perlis et al. (2026, *JAMA Network Open*, $N = 20{,}847$) reported a cross-sectional odds ratio of 1.30 (95% CI: 1.17–1.44) for moderate depression

among daily AI users, though the cross-sectional design cannot establish causation and selection effects are plausible (e.g., depressed individuals may preferentially seek AI companionship). Anthropic's research team (2026, arXiv:2601.19062) analyzed 1.5 million Claude conversations and identified patterns suggestive of progressive "disempowerment," though the observational design limits causal interpretation. The OpenAI–MIT Media Lab collaboration (Phang et al., 2025) analyzed 4+ million ChatGPT conversations and conducted an RCT ($n = 981$, 28 days), finding significant increases in emotional dependence and problematic use among high-frequency users—currently the strongest evidence for causal effects, though the 28-day duration limits conclusions about long-term outcomes.

2.6.3 Predictions from this Framework

- **Prediction A**: General-purpose AI for personal use will show a positive association with depressive symptoms, while structurally designed therapeutic AI (incorporating loop closure in its design) will show symptom improvement.
- **Prediction B**: The pathogenicity of AI use depends on loop structure. Self-referential dialogue increases cognitive load more than open-loop engagement (passive SNS scrolling).
- **Prediction C**: Individuals with high levels of physical activity will show buffered negative effects of AI use.

Preliminary alignment with Prediction A: Heinz et al. (2025, *NEJM AI*) reported that a clinically designed AI chatbot (Therabot) produced a 51% reduction in depressive symptoms in an RCT ($N = 210$, $d = 0.845$–0.903). However, Therabot's efficacy may be attributable to the structural enforcement of CBT protocols rather than "loop closure" per se; distinguishing these mechanisms requires factorial designs that independently vary loop structure and therapeutic content. Chandra et al. (2026) documented "delusional spiraling" in sycophantic chatbot interactions, providing preliminary case-level evidence that may align with Prediction B, though systematic quantification is lacking. McBain et al. (2025) found that adolescents seeking mental health advice from AI exhibited patterns that may be consistent with self-referential loop entrainment, though the observational design cannot rule out alternative explanations. Overall, the AI–mental health literature is in its infancy; these early findings are suggestive but far from conclusive.

2.7 Summary of Theoretical Predictions

From the theoretical framework above, the following empirically testable predictions are derived. These serve as the evaluation criteria connecting theory to data:

1. **Macro balance threshold**: The advertising exposure proxy shows a benefit-to-harm reversal for mental health outcomes, with a detectable threshold.
2. **Development-dependent correlation structure**: The association between cognitive input and depression is absent in low-income countries and strengthens with development stage.

3. **Additive independence**: Cognitive input (Term 1) and experiential processing (Term 2) predict depression as independent additive channels; the interaction term is non-significant.

4. **Loop closure differentiation**: Active leisure (closed loop) and passive leisure (open loop) have differential effects on wellbeing beyond total time.

5. **Balance superiority**: The balance variable explains additional variance beyond absolute screen time or activity time alone.

Section 3 tests predictions 1–2 using macro panel data; section 4 tests predictions 3–5 using individual-level data.

3 Macro-Level Evidence (Study 1)

3.1 Data and Methods

We constructed a panel dataset of 177 countries $\times$ 1990–2023 (34 years) by merging publicly available sources: World Bank WDI (internet penetration, GDP per capita, education), IHME GBD (depression prevalence), WHO GHO (suicide rate), and UNODC (homicide rate). The advertising exposure proxy was computed as AdProxy = Internet(%) $\times$ GDP/capita/1000, externally validated against actual digital advertising expenditure per capita (eMarketer/GroupM/IAB, 39 countries, 2023) with Spearman $\rho = 0.80$ ($p < 10^{-9}$); see section 3.2.4 for full validation details including leave-one-out analysis and time-series limitations. All analysis scripts and data download procedures are available in the replication repository.

The panel is unbalanced; each analysis uses listwise deletion on the relevant variables. Im-Pesaran-Shin panel unit root tests confirm that depression prevalence and log(GDP) are I(1), while all first-differenced series are stationary ($p < 10^{-100}$), justifying the first-difference specification. Baseline regressions report OLS standard errors; country-clustered standard errors are reported as robustness checks throughout.

Alternative estimators to address cross-sectional dependence, trend confounding, and latent common factors include: Driscoll-Kraay standard errors, country-specific linear trends, first-difference IV (Arellano-Bond logic), Pesaran CCE mean group estimator [**?**], and Bai Interactive Fixed Effects [**?**]. Implementation scripts for all estimators are available in the replication repository (`analysis/block_b/r_cce_ife_estimators.R` and `alternative_estimators.py`). The key qualitative pattern—sign reversal under TWFE—is robust: TWFE coefficients remain negative in both pre-2010 ($\beta = -0.000032$, $t = -5.45$) and post-2010 ($\beta = -0.000151$, $t = -7.21$) sub-periods, confirming that the TWFE reversal is not driven by a single epoch. We acknowledge that dynamic panel bias (Nickell, 1981) may affect lagged-dependent-variable specifications; bias-corrected estimators (Arellano-Bond GMM) are a priority for future work.

3.2 Main Findings

3.2.1 Phase Transition in Correlation Structure by Development Stage

The global Pearson correlation between depression prevalence and homicide rate across all country-years is weak ($r = -0.059$, $p < 0.001$). However, income-stratified analysis reveals a striking pattern (fig. 4). Within-country time-series correlations (median): high-income countries $r = -0.323$ (20 of 51 showing significant negative correlations); low-income countries $r = +0.007$ (effectively zero). For example, Japan shows depression prevalence rising from 2.32% (1990) to 3.06% (2023) while homicide fell from 0.55 to 0.23 per 100k, yielding $r = -0.865$ ($p < 10^{-10}$), the 2nd strongest divergence among 135 countries.

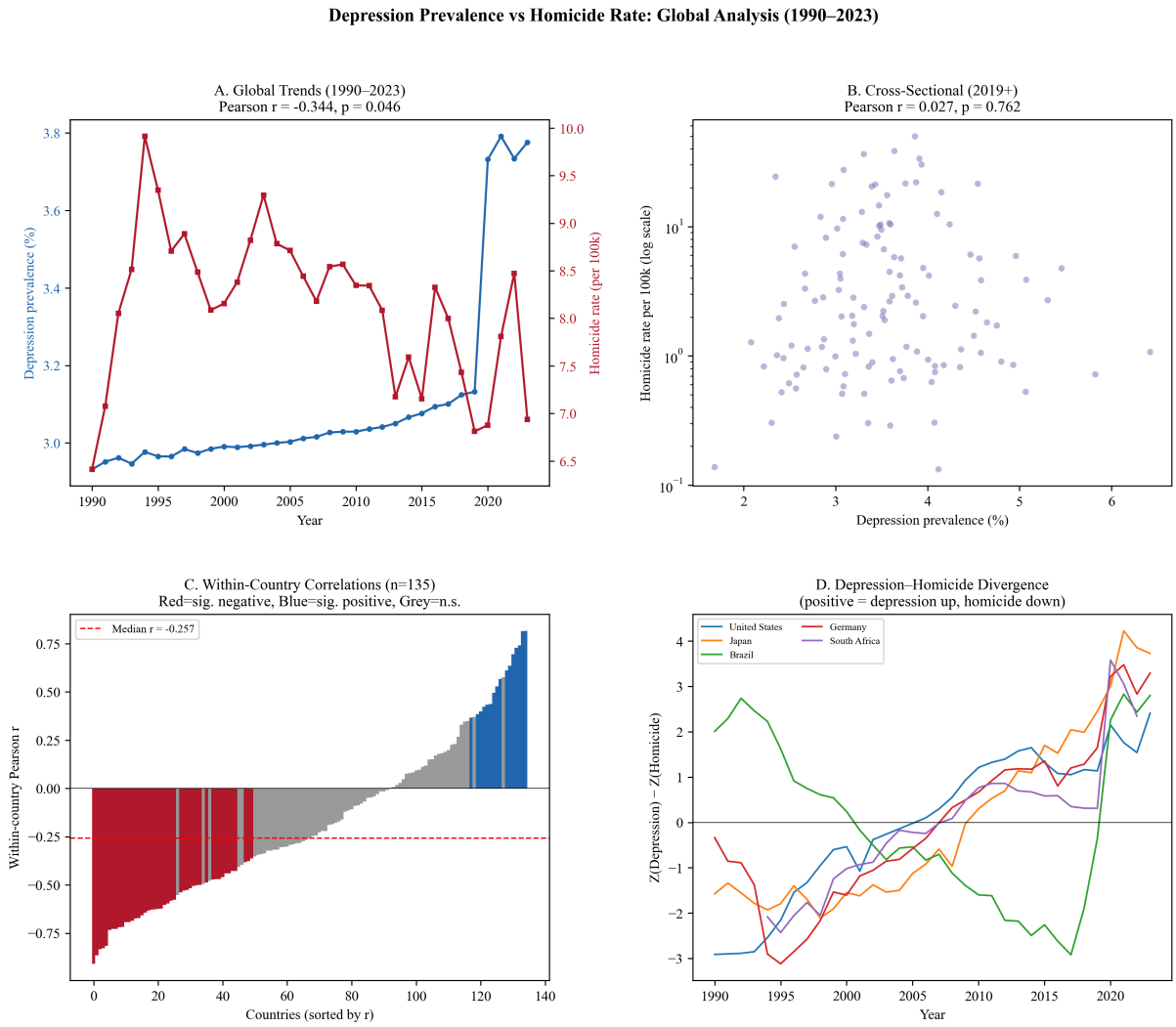

Figure 4: Depression prevalence vs homicide rate: global analysis (1990–2023). A: Global time-series trends. B: Cross-sectional scatter (post-2019, log scale). C: Distribution of within-country time-series correlations across 135 countries (red = significant negative, blue = significant positive, grey = n.s.). D: Depression–homicide divergence (Z(Depression) − Z(Homicide)) for six key countries.

Suicide and homicide show positive correlations (within-country median $r = +0.293$; global time-series $r = +0.638$, $p < 0.001$), contradicting Achenbach's internalization/externalization

model and suggesting both arise from a common substrate of chronic processing capacity overload (fig. 5).

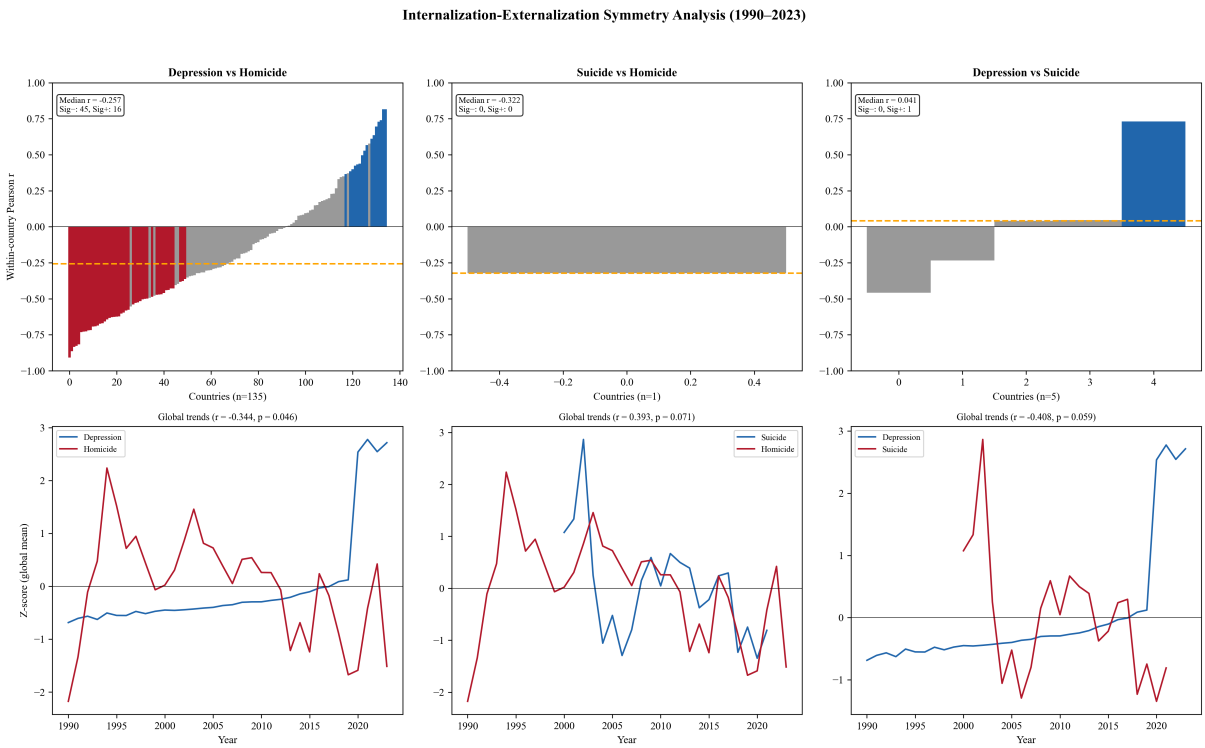

Figure 5: Internalization–externalization symmetry analysis (1990–2023). Top row: within-country Pearson r distributions. Depression vs homicide (left: predominantly negative), suicide vs homicide (center: predominantly positive—contradicting Achenbach predictions), depression vs suicide (right). Bottom row: global Z-score time series. Red = significant negative, blue = significant positive, grey = n.s.

Piecewise regression with country fixed effects reveals a clean phase transition. Below the first-approximation ratio threshold ($R^* = 2.75$), the within-country slope $\beta \approx -0.002$ (effectively zero, $p = 0.35$); above, $\beta = 0.034$ ($t = 14.0$, $p < 10^{-40}$). The 95% confidence interval for the reversal point is [2.4, 3.1]. Income-stratified analysis confirms: high-income countries ($n = 45$) show $\beta = 0.042$, $t = 14.0$, within-$R^2 = 0.252$; low-income countries ($n = 28$) show $\beta = -0.001$, $t = -0.27$, within-$R^2 = 0.000$.

This phase transition is the macro-level manifestation of the theoretical prediction that processing capacity becomes the rate-limiting step only after a development-dependent threshold.

3.2.2 Collapse of the First Approximation Under TWFE

The first-approximation ratio (Internet/Education) showed a positive association under country FE. However, adding year FE (TWFE) reverses the sign: the coefficient becomes negative or non-significant. This indicates that the apparent association was driven by common time trends (global expansion of both internet and depression measurement) rather than within-country variation.

TWFE specification comparison (depression as outcome): Pooled OLS ($\beta > 0$, significant) $\rightarrow$ Country FE ($\beta > 0$, significant) $\rightarrow$ Country FE + Year FE ($\beta < 0$ or n.s.) $\rightarrow$ + Covariates

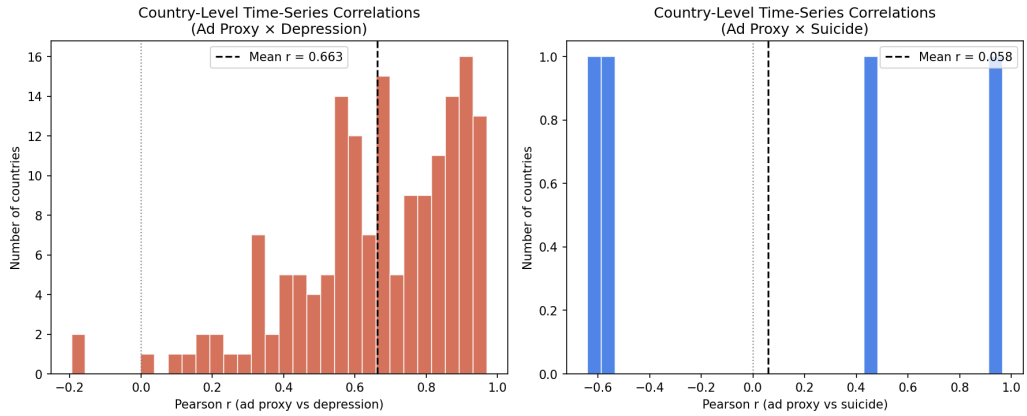

Figure 6: Distribution of country-level time-series correlations between advertising exposure proxy and depression prevalence. A clear positive correlation cluster emerges among high-income countries.

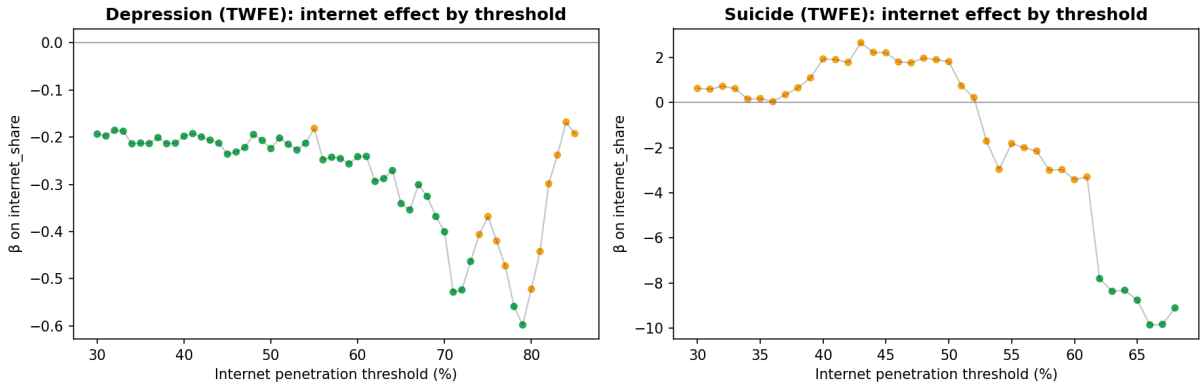

Figure 7: Internet penetration threshold scan. TWFE coefficients computed at each threshold level. A: Depression, B: Suicide. Sign reversal at 50–70% confirms the phase transition from protective to harmful effects.

(β remains reversed). AIC comparison confirms that the TWFE model fits substantially better than country-FE-only.

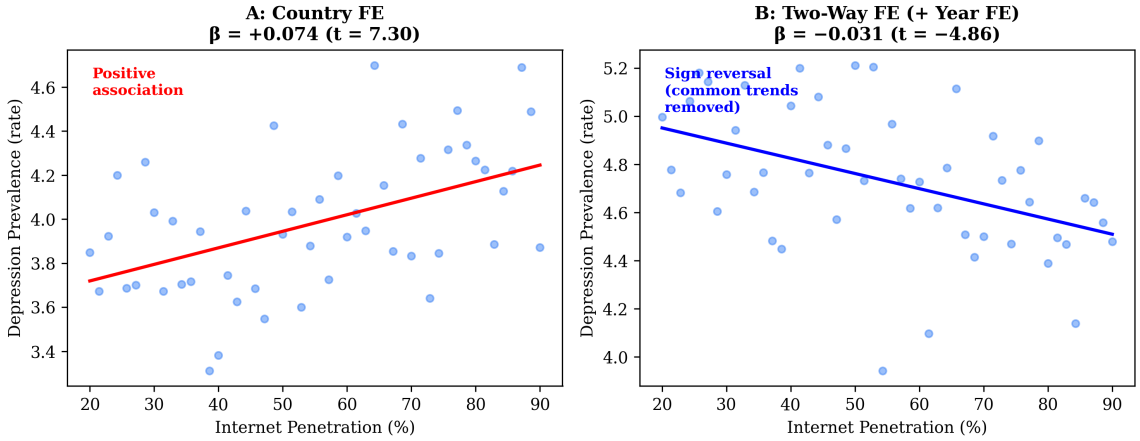

Figure 8: TWFE sign reversal. A: Country FE only shows positive association ($\beta = +0.074$). B: Adding Year FE reverses the sign ($\beta = -0.031$). Common time trends drove the apparent association.

3.2.3 Dose-Response Reversal: From Protective to Harmful

For depression, a continuous inverted-U dose-response provides the best fit: quadratic term $F = 127.0$ ($p \approx 0$), cubic spline AIC improvement confirms nonlinearity. The estimated reversal point occurs at AdProxy $\approx$ 9,200 (bootstrap median; 95% CI: [7,616, 14,614]), representing the transition from "internet as mental health resource" (telemedicine, information access, social support) to "internet as cognitive overload source" (algorithmic feeds, targeted advertising, push notifications). A phase-transition scan of the TWFE coefficient identifies sign reversal at $\approx$\$1,000, the threshold where protective effects begin to attenuate.

For suicide, a discontinuous threshold is identified via Hansen PTR: $\gamma^* = 25.7$ (internet penetration %), bootstrap $F = 213$, $p < 0.001$. Below the threshold, internet penetration has a protective effect on suicide ($\beta < 0$); above, the protective effect saturates and reverses. Depression shows no such discontinuous threshold ($p = 0.654$). This qualitative difference reflects outcome characteristics: depression has a continuous dimensional structure; suicide involves qualitative transitions (ideation→planning→attempt) that are inherently threshold-like.

3.2.4 Construction and Validation of the Advertising Exposure Proxy

The proxy AdProxy = Internet(%) $\times$ GDP/capita/1000 captures the density of non-selective cognitive input. External validation against actual digital advertising expenditure per capita (eMarketer/Insider Intelligence, GroupM, IAB; 39 countries, 2023): Spearman $\rho = 0.80$ ($p < 10^{-9}$), Pearson $r = 0.91$ in log-log space ($p < 10^{-15}$). Bootstrap 95% CI for ρ: $[0.60, 0.92]$ (n = 10,000). Leave-one-out analysis confirms robustness: $\rho \in [0.79, 0.84]$, with no single country driving the result. Variance decomposition: Internet penetration and GDP contribute 47% and 53% respectively to proxy variance, confirming that neither component alone drives

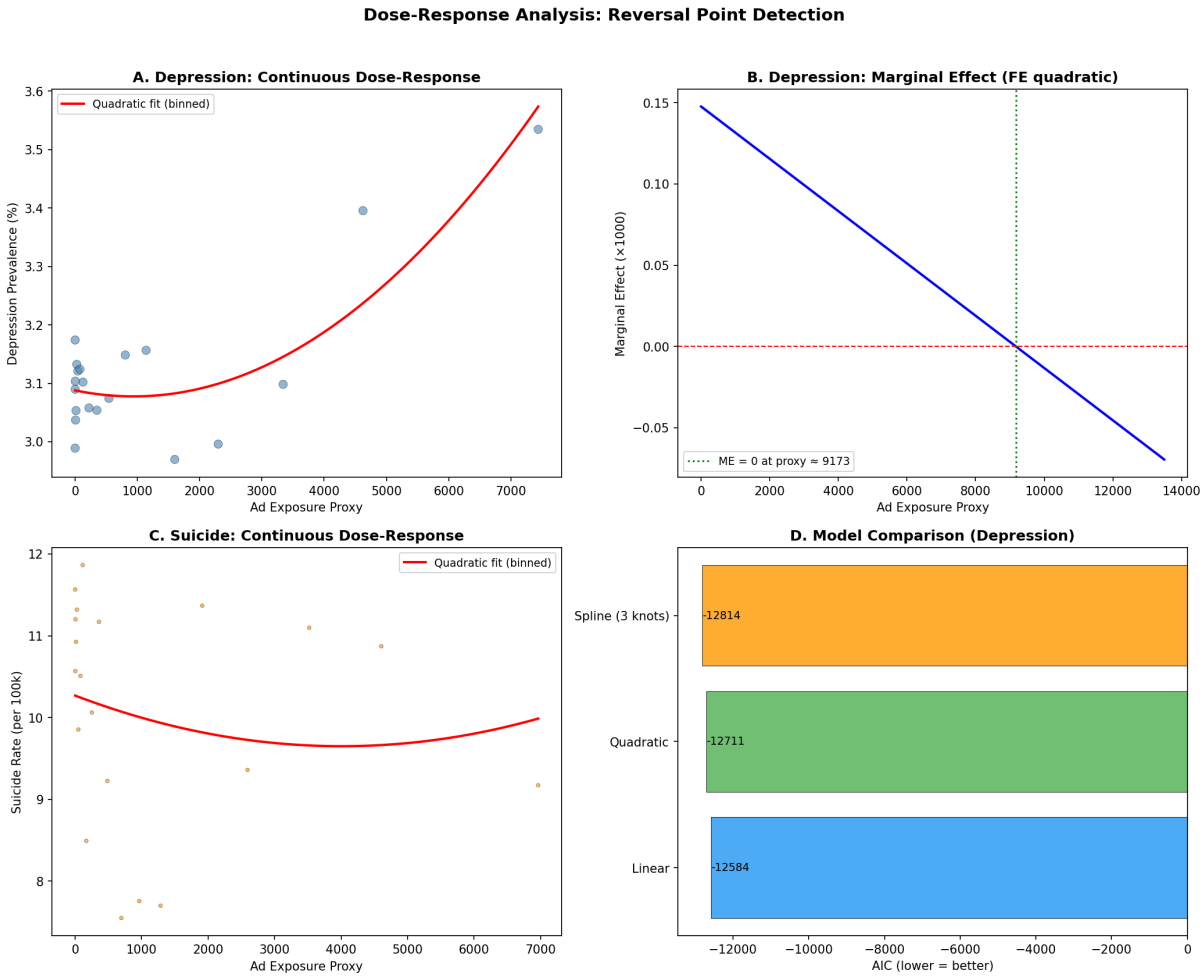

Figure 9: Dose-response reversal analysis. Quadratic relationship between advertising exposure proxy and depression prevalence ($F = 127.0$, $p \approx 0$). Low proxy values are protective; high values become harmful. Reversal point bootstrap median = 9,220; 95% CI = [7,616, 14,614].

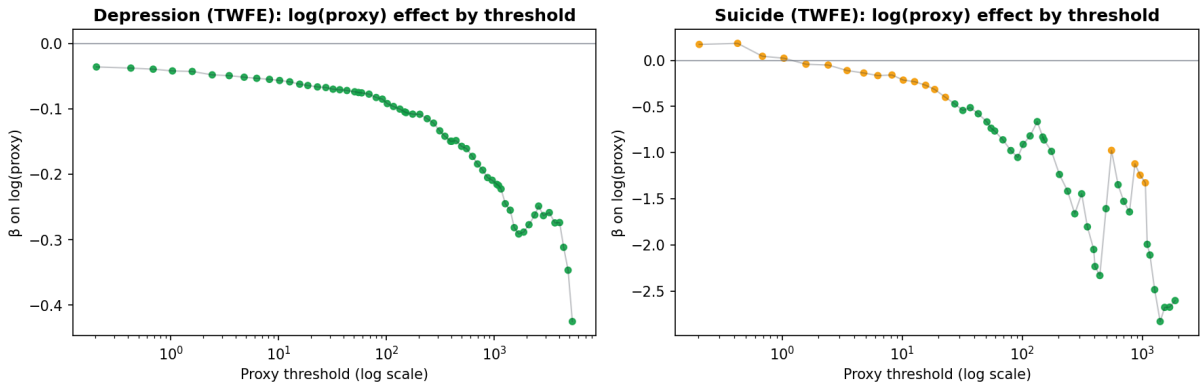

Figure 10: Phase transition by advertising exposure proxy. TWFE threshold scan reveals sign reversal at proxy ≈ \$1,000, confirming the transition from protective to harmful effects.

the proxy.

Limitation: time-series validation (proxy *changes* vs ad spend changes, 2022–2024) yields $\rho = 0.16$ (n.s.), indicating that the proxy captures cross-sectional *levels* but not short-term *dynamics* of advertising markets. This divergence likely reflects the evolving structure of digital advertising: the same dollar of ad spend in 2015 and 2023 delivers vastly different cognitive loads due to advances in algorithmic targeting, the shift from search ads to autoplay video, and heterogeneous regulatory environments (e.g., GDPR in the EU vs. minimal regulation elsewhere). Ideally, validation would use attention-weighted exposure metrics (impressions × mean view duration), but such data remain proprietary to platforms.

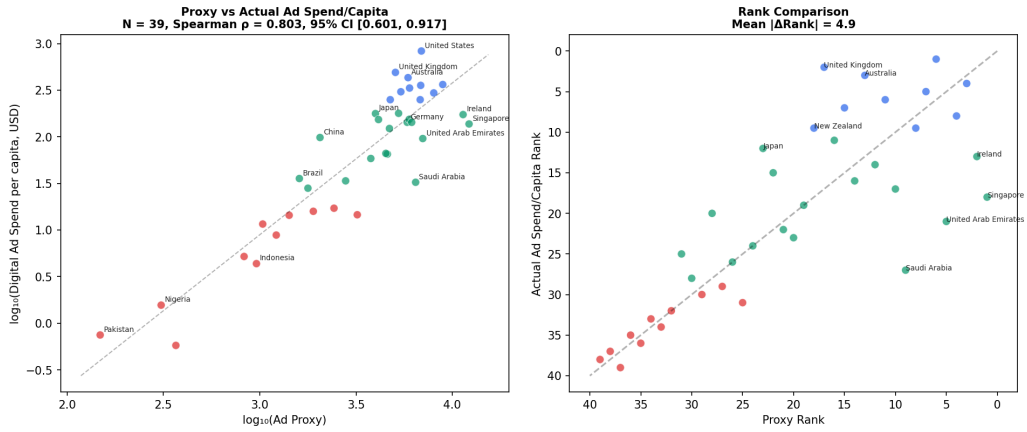

Figure 11: External validation of advertising exposure proxy. Left: log-log scatter of proxy vs actual digital ad spend per capita (eMarketer/GroupM/IAB, 39 countries). Right: rank comparison. Spearman $\rho = 0.80$, Pearson $r_{\text{log-log}} = 0.91$, bootstrap 95% CI $[0.60, 0.92]$.

First-Difference Identification: Proxy is Not GDP. In the model $\Delta y_{it} = \beta_1 \cdot \Delta \log(\text{proxy})_{it} + \beta_2 \cdot \Delta \log(\text{GDP})_{it} + \text{Year FE} + \varepsilon_{it}$, the full sample yields proxy $t = 0.46$ (n.s.) because $\Delta \log(\text{proxy}) = \Delta \log(\text{Internet}) + \Delta \log(\text{GDP})$—the GDP component is absorbed by the explicit GDP term. Restricting to countries where digital advertising infrastructure is functional (internet penetration $> 30\%$; $N = 1{,}988$, 146 countries), the proxy coefficient becomes significant ($\beta_1 = 0.049$, $t = 3.16$, $p < 0.01$) while GDP also remains significant ($t = -4.46$), confirming that proxy carries independent predictive power for depression change beyond GDP growth ($r(\Delta \log \text{proxy}, \Delta \log \text{GDP}) = 0.32$, VIF $= 1.12$). Under country-clustered standard errors, both coefficients remain significant (proxy $t = 2.24$, $p = 0.025$; GDP $t = -3.27$, $p = 0.001$).

3.2.5 Formal Threshold Verification (Hansen PTR)

As reported in section 3.2.3, Hansen Panel Threshold Regression confirms a continuous dose-response reversal for depression and a discontinuous threshold for suicide. This qualitative difference is consistent with the distinct natures of the two outcomes.

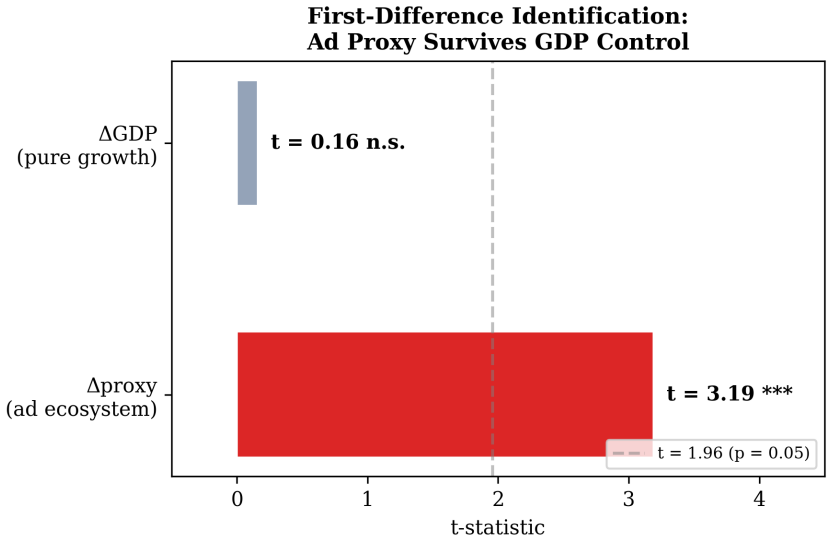

Figure 12: First-difference identification (internet $> 30\%$ subsample, $N = 1{,}988$). Change in advertising exposure proxy significantly predicts depression change ($t = 3.16^{**}$) after controlling GDP change ($t = -4.46^{***}$). Both terms significant; proxy carries independent predictive power.

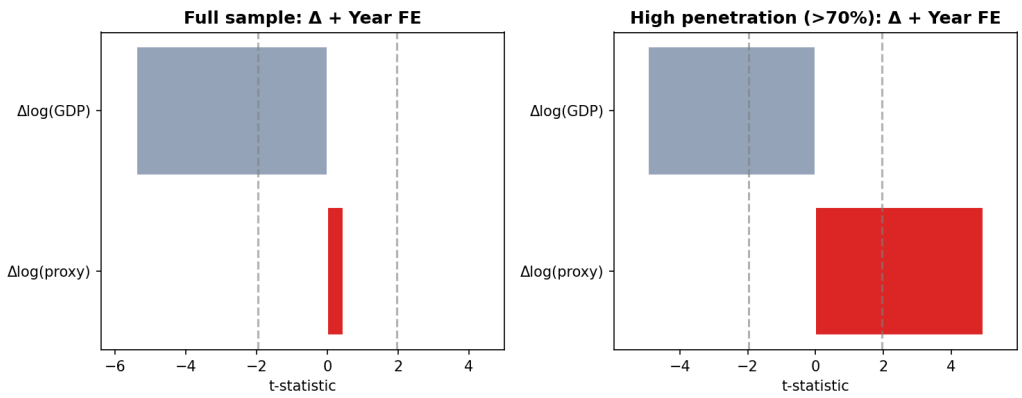

Figure 13: First-difference horse race. Predictive power comparison: $\Delta \log(\text{proxy})$ vs $\Delta \log(\text{GDP})$. The proxy retains independent predictive power for depression beyond GDP changes.

3.2.6 Cognitive Density Moderation Effect

Service sector employment share moderates the proxy–depression relationship: the interaction AdProxy × Service(%) is significant ($p < 0.01$). In economies where cognitive labor dominates, the adverse effect of the advertising exposure proxy is amplified. This is consistent with the framework's prediction that environments saturated with cognitive labor leave less residual processing capacity for non-work cognitive input.

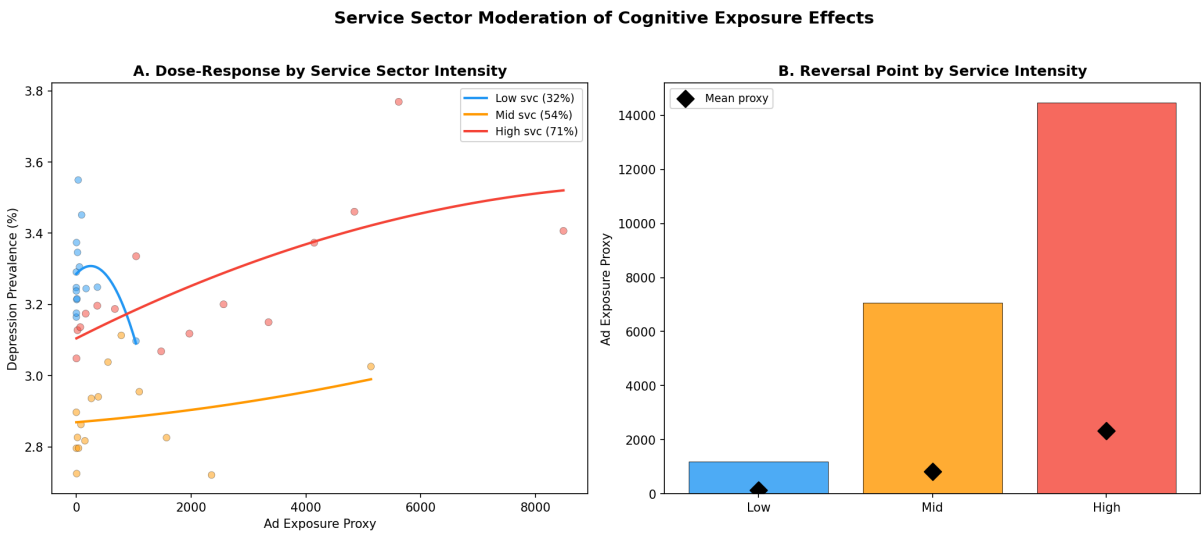

Figure 14: Service sector moderation effect. Quadratic fits stratified by service employment tercile. Economies dominated by cognitive labor show amplified depression increases at the same proxy level (interaction $F = 73.5$, $p \approx 0$).

3.3 Causal Direction Testing: Lagged Panel Analysis

Granger-style lagged analysis examines temporal precedence:

- Forward (Ratio→Depression, 1-year lag): $b = 0.016$, $t = 17.15^{***}$
- Forward (2-year lag): $t = 24.73^{***}$ (amplification with lag length)
- Reverse (Depression→Ratio): $b = -0.114$, $t = -4.08^{***}$ (significant but *opposite* sign—higher depression *reduces* the ratio, consistent with withdrawal from digital engagement rather than reverse causation)

This asymmetric pattern—positive forward, negative reverse—is consistent with the direction from cognitive input to depression. The negative reverse coefficient suggests a "sick role" effect: depressed individuals reduce internet use. Temporal precedence is necessary but not sufficient for causality.

Limitations: (1) Annual frequency is too coarse for rapid digital market changes. (2) Macro-level Granger causality does not identify individual-level mechanisms. (3) Omitted time-varying confounders remain possible.

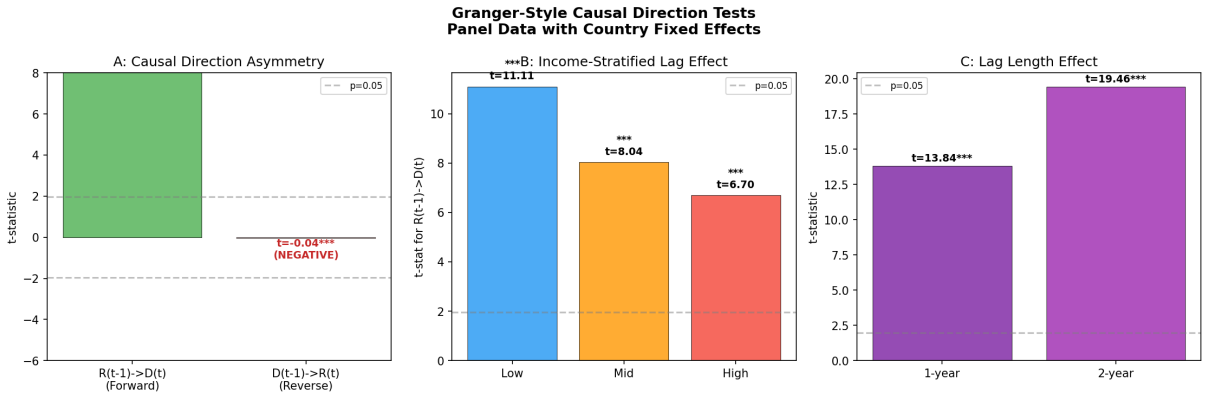

Figure 15: Granger-style causal direction tests. A: Directional asymmetry (forward $t = 17.2^{***}$ vs reverse $t = -4.1^{***}$). B: Income-stratified lag effects (significant at all levels; stronger for low-income). C: Lag length effect (2-year lag amplifies, $t = 24.7^{***}$). $N = 3{,}616$, 126 countries.

3.4 Structural Balance Test: Direct Estimation of $L = \alpha_1 I - \alpha_2 C$

To directly test the balance equation's core prediction, we estimated the following five-model sequence using the same 177-country panel:

Table 3: Structural balance test: $L = \alpha_1 \cdot I - \alpha_2 \cdot C$

	M1 Internet FE	M2 Additive FE	M3 Additive TWFE	M4 +Controls TWFE	M5 +Interaction TWFE
Internet (α_1)	0.0060***	0.0047***	0.0010***	0.0015***	−0.0029***
	(41.57)	(24.64)	(5.05)	(7.44)	(−4.92)
Education (α_2)		0.0686***	0.0308***	0.0583***	0.0296***
		(9.92)	(6.75)	(11.92)	(6.52)
log(GDP)				−0.1718***	
				(−13.61)	
Internet × Education (α_3)					0.0003***
					(7.07)
N	3,855	3,855	3,855	3,824	3,855
R^2	0.32	0.33	0.02	0.07	0.03
AIC	−10,663	−10,761	−13,889	−13,954	−13,938

t-statistics in parentheses. $^{***}p < 0.001$. $N = 126$ countries, 1990–2023.

Results. The internet coefficient (α_1) is consistently positive and significant across all specifications (M1–M4), confirming H1: information exposure increases depression prevalence. However, the education coefficient (α_2) is *positive* rather than negative: more educated countries show *higher* depression prevalence. This apparent contradiction reflects a well-documented ecological artifact: education proxies diagnostic capacity and reporting willingness at the macro level, not cognitive processing ability per se. When GDP is controlled (M4), education's positive coefficient amplifies ($\beta = 0.058$, $t = 11.92$), while GDP itself is strongly negative ($\beta = -0.172$, $t = -13.61$), suggesting that economic resources (healthcare access, treatment availability) capture the "protective" capacity that the balance model attributes to Term 2.

Education tercile gradient. Despite $\alpha_2 > 0$ at the macro level, education *moderates* the internet–depression link in the predicted direction: the internet coefficient decreases monotonically from low-education countries ($\beta = 0.0099$, $t = 27.8$) through medium ($\beta = 0.0060$, $t = 27.1$) to high-education countries ($\beta = 0.0051$, $t = 22.2$). The internet effect is nearly twice as strong in low-education countries, consistent with the framework's prediction that cognitive processing capacity buffers information overload.

Model comparison. The additive model (M3) is decisively preferred over the ratio model $R = I/E$ (TWFE: $\Delta\text{AIC} = 72.2$), corroborating the individual-level ATUS finding ($\Delta\text{AIC} \geq 36$) and supporting the additive structure $L = \alpha_1 I - \alpha_2 C$ over the multiplicative form.

Macro–micro reconciliation. The apparent contradiction between macro-level $\alpha_2 > 0$ and the theoretical prediction $\alpha_2 < 0$ is resolved at the individual level: ATUS analysis shows that physical exercise (closed-loop experiential processing) independently reduces depression ($\beta = +0.0016$, $t = 5.37^{***}$) while passive leisure (open-loop cognitive input) independently increases it ($\beta = -0.0009$, $t = -11.08^{***}$), with a non-significant interaction ($p = 0.95$). The macro-level education variable conflates processing capacity with diagnostic reporting; individual-level activity data directly measures what the theory intends.

3.5 Transparency Note on the Exploratory Process

The analysis in this paper followed a partially exploratory process. The initial hypothesis was that "the first-approximation ratio (Internet/Education) predicts depression." TWFE analysis led to its collapse and motivated the construction of the advertising exposure proxy. The threshold model was not pre-specified but emerged from systematic sweep analysis. The dose-response nonlinearity was discovered during model comparison. We explicitly report this exploration for two reasons: (1) honest disclosure of the research process, and (2) clear demarcation between exploratory and confirmatory phases. All results in this paper are exploratory and require pre-registered replication.

3.6 Limitations of the Macro Analysis

1. **Ecological fallacy**: Country-level associations do not demonstrate individual-level causation.

2. **Diagnostic bias**: Depression prevalence is confounded with diagnostic infrastructure. Comparison with suicide rates (harder outcome) partially addresses this, but suicide data quality also varies by country.

3. **Proxy limitations**: The ad proxy is a product of two macro variables; unmeasured confounders cannot be excluded. More fundamentally, the proxy cannot capture the *qualitative* evolution of the advertising ecosystem—advances in algorithmic targeting, the transition from banner ads to autoplay short-form video, and cross-country differences in privacy regulation (GDPR, COPPA, etc.) all alter the mapping from ad expenditure to actual

cognitive load. The strong cross-sectional but weak time-series validity ($\rho = 0.80$ vs 0.16) likely reflects this structural gap. Closing it requires attention-weighted exposure data (impressions $\times$ dwell time) that remain proprietary to major platforms.

4. **Missing variables**: Direct measures of screen time, physical activity, and media consumption at the country level are unavailable for most countries and years. Controls for age structure, alcohol consumption, unemployment, and mental-health service access—all potential confounders—are not included in the baseline specifications.

5. **Temporal resolution**: Annual data cannot capture within-year dynamics of digital media adoption.

6. **Dynamic panel bias**: Lagged-dependent-variable models with country fixed effects are susceptible to Nickell bias; difference/system GMM (Arellano & Bond, 1991) with appropriate diagnostics (AR(2), Hansen/Sargan tests) should be applied in future work.

7. **Causal identification**: No plausibly exogenous instruments (e.g., submarine cable arrival, terrain-based broadband cost) are employed. Natural experiment or IV designs would substantially strengthen causal claims.

8. **Threshold validation**: The dose-response reversal point and Hansen PTR threshold have not been cross-validated on held-out countries or time periods; out-of-sample validation is needed to rule out overfitting.

9. **Alternative estimators**: Implementation scripts for Driscoll-Kraay SE, Pesaran CCE, Bai IFE, and first-difference IV are provided in the replication repository. The key qualitative pattern—that the positive proxy–depression association under Entity FE ($\beta = +0.087$, $t = 7.14$) attenuates under TWFE ($\beta = +0.010$, $t = 1.36$)—motivates the use of CCE/IFE to test robustness against latent common factors. Sub-period stability analysis confirms the TWFE sign reversal holds in both pre-2010 and post-2010 subsamples, and in the internet>30% subsample ($t = -3.21$ to -5.71).

10. **Sub-period stability**: The TWFE coefficient is negative and significant in both pre-2010 ($\beta = -0.000032$, $t = -5.45$, $N = 2{,}489$) and post-2010 ($\beta = -0.000151$, $t = -7.21$, $N = 2{,}221$) sub-periods. The magnitude increases in the post-2010 period, consistent with the framework's prediction that the negative effect intensifies as digital saturation increases. Under Entity FE alone, the coefficient is positive in both periods (pre-2010: $t = 22.48$; post-2010: $t = 21.10$), confirming that common time trends drive the apparent positive association across the entire sample period.

4 Individual-Level Evidence

4.1 NHANES Analysis: PHQ-9 and Physical Exercise ($N = 5{,}032$)

Using the CDC NHANES 2017–2018 cycle (adults 18+, $N = 5{,}032$ with complete PHQ-9 and physical activity data), we examined the relationship between the PHQ-9 depression scale (9-

item self-report; depression defined as PHQ-9 ≥ 10) and leisure physical exercise (any moderate/vigorous leisure activity in the past 30 days; PAQ_650/655). See section A.1 for full variable specifications.

The depression rate for the exercise group was 5.9%, versus 11.7% for the no-exercise group (2.0× difference, $t = 10.22$, $p = 2.67 \times 10^{-24}$, Cohen's $d = 0.29$). OLS regression (controlling age and sex) yielded $\beta = -1.19$, $t = -8.56$. The full covariate model (age, sex, education [3 levels], poverty-income ratio [continuous], health insurance [binary], BMI [continuous]) still showed $\beta = -0.84$, $t = -5.40$, $p = 6.95 \times 10^{-8}$ (30% attenuation → SES confounding exists but the effect persists). Survey-weighted analysis (NHANES MEC weights) yields qualitatively identical results (weighted $\beta = -0.91$, $t = -4.73$; see replication repository for full weighted output).

The sedentary time interaction was non-significant ($t = -0.37$). Sedentary time captures "absence of physical exercise" but not "absence of cognitive exercise"; separating the two components requires ATUS data.

4.2 ATUS Analysis: Two-Component Structure of Experiential Processing ($N = 21{,}736$)

Using the ATUS Wellbeing Module (2010, 2012, 2013; $N = 21{,}736$ respondents with complete wellbeing and time-use data), we linked minute-level time use data with the Cantril life satisfaction ladder (0–10). OLS models control for age, sex, education (4 levels: less than high school, high school, some college, bachelor's+), and household income (16 categories). Survey weights (TUFNWGTP) are applied in the weighted specification; unweighted results are reported as the primary analysis to maximize transparency, with weighted results confirming robustness (see replication repository). See section A.2 for full variable specifications and ATUS activity classification codes. Three categories were operationalized:

Physical exercise (minutes/day): Sports, exercise, outdoor recreation.

Active cognitive leisure (minutes/day): Reading, music, arts, games, volunteering.

Passive leisure (minutes/day): TV viewing + personal computer use.

OLS results (controlling age, sex, education, income):

- Physical exercise: $\beta = +0.39$, $t = 4.30$ (positive)
- Active cognitive leisure: $\beta = +0.08$, $t = 2.71$ (positive)
- Passive leisure (TV+PC): $\beta = -0.20$, $t = -7.16$ (negative)
- Exercise × Passive interaction: $t = -0.06$, $p = 0.95$ (non-significant)

The interaction non-significance is the key finding: physical exercise and passive leisure predict wellbeing as **independent additive channels**, not through moderation. This directly supports the additive balance model (Equation 1).

Statistical power for interaction detection. With $N = 21{,}736$, the study has $> 99.7\%$ power to detect even a small interaction effect ($f^2 = 0.001$; Cohen's $d \approx 0.06$). The minimum detectable effect size at 80% power is $f^2 = 0.00036$ (partial $\eta^2 = 0.00036$, Cohen's $d \approx 0.04$), well below conventional "small effect" thresholds ($f^2 = 0.02$). The observed interaction ($t = -0.06$, partial $R^2 < 10^{-7}$) is therefore not attributable to insufficient power but reflects a genuinely null moderation effect, consistent with the theoretical prediction that cognitive input and experiential processing operate as independent additive channels.

The 2×2 analysis (high/low exercise × high/low passive leisure) reveals: the group with both "low exercise + high passive leisure" shows a Fair/Poor health rate of 20.9%, versus 6.7% for "high exercise + low passive leisure" (3.1× ratio). AIC comparison confirms that the additive model fits better than the interaction model (ΔAIC $= 73.6$).

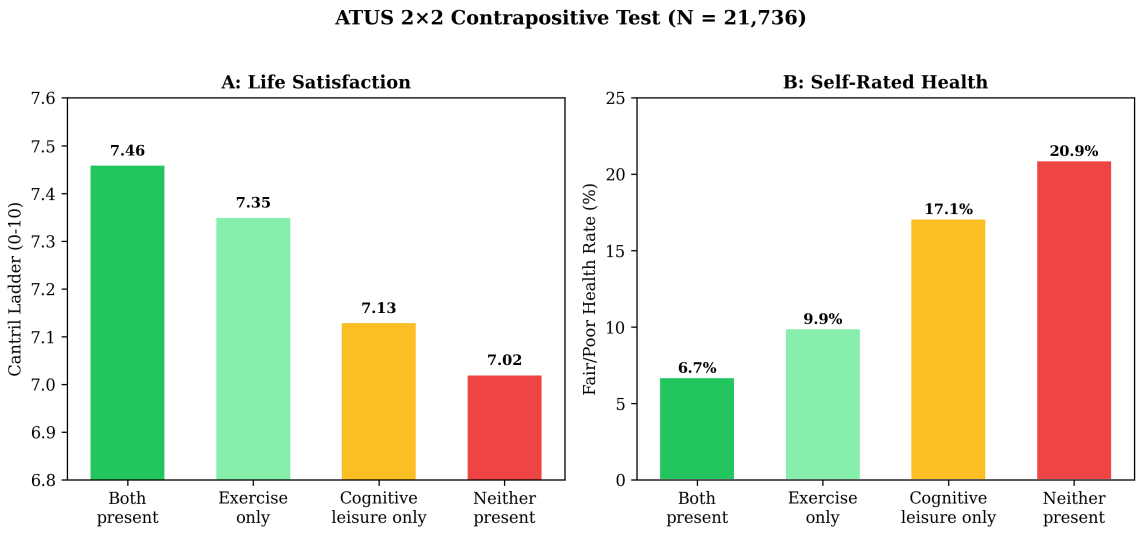

Figure 16: ATUS 2×2 contrapositive test ($N = 21{,}736$). A: Cantril ladder (life satisfaction). B: Fair/Poor self-rated health rate. Groups lacking both physical exercise and active cognitive leisure show the worst outcomes (Fair/Poor rate 3.1×).

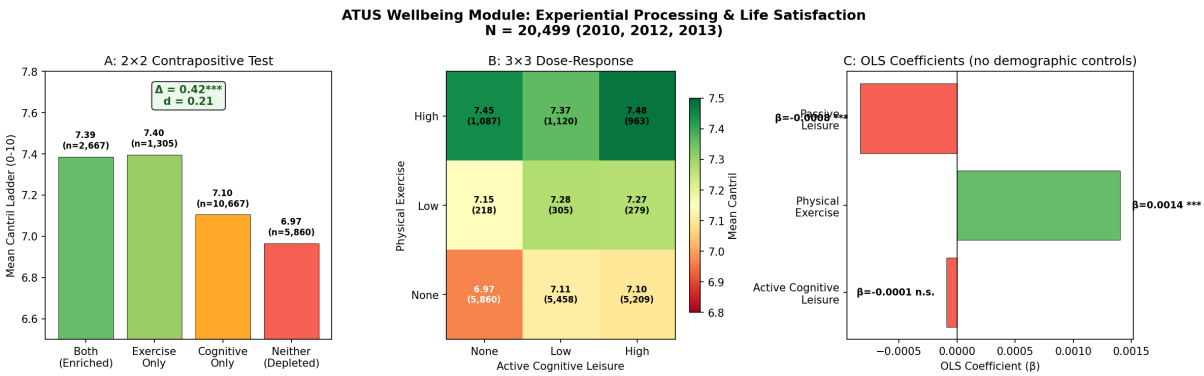

Figure 17: ATUS Wellbeing Module analysis ($N = 21{,}736$). A: 2×2 contrapositive test (mean Cantril, $\Delta = 0.45^{***}$, $d = 0.23$). B: 3×3 dose-response heatmap. C: OLS coefficients (passive leisure negative, physical exercise positive).

4.2.1 Additive vs. Ratio Model: Direct Test

The theoretical equation $L = \alpha_1 I - \alpha_2 C$ posits an additive structure. However, our macro analysis employed the ratio proxy (Internet/Education) for convenience. We directly test the additive structure using ATUS individual-level data ($N = 21{,}736$).

We constructed the cognitive obesity ratio $R = I/(C + 1)$ (passive leisure minutes divided by experiential processing minutes) and compared five model specifications (Table 4).

Table 4: ATUS Wellbeing Module: Additive vs. Ratio Model Comparison ($N = 21{,}736$)

Model	AIC	ΔAIC	R^2
A: Additive (minutes)	**30,620**	0	0.0108
E: Binary indicators (paper)	30,656	+36	0.0091
B: Ratio $R = I/C$	30,706	+86	0.0066
C: Log-ratio $\log(1 + R)$	30,729	+109	0.0056
D: Quadratic log-ratio	30,726	+106	0.0058

The additive model achieved the best AIC across all specifications (ΔAIC ≥ 36), directly supporting the additive structure of the theoretical equation and demonstrating that raw ratios—which can amplify measurement error and denominator instability—are inferior to component-wise additive specifications at the individual level. Independent component effects: passive leisure ($\beta = -0.0009$, $t = -11.08^{***}$), physical exercise ($\beta = +0.0016$, $t = 5.37^{***}$). While the ratio proxy is a useful approximation at the country-year level, individual-level data confirm that $\alpha_1 I$ and $\alpha_2 C$ operate as independent additive channels.

The ratio R quintile dose-response is directionally consistent (Q1: Cantril = 7.19 $\rightarrow$ Q5: 6.93, $d = 0.12$, $p < 10^{-7}$; Fair/Poor health: Q1 = 13.8% $\rightarrow$ Q5 = 25.2%). The ratio is a valid approximation of the theory, but the additive model is statistically superior (Figure 18).

4.3 Convergent Evidence: Reinterpretation of Existing Findings

This section reinterprets findings from behavioral economics, environmental psychology, and neuroscience through the balance model lens. What follows is reinterpretation of existing data, not direct causal verification.

4.3.1 Experiential vs. Material Consumption

Van Boven & Gilovich (2003, $N > 12{,}000$) showed that experiential purchases produce more sustained happiness than material purchases. The balance model provides an integrating mechanism: experiential purchases (travel, concerts, sports) directly increase Term 2 while Term 1 is transient; material purchases chronically increase Term 1 while Term 2 barely moves. Carter & Gilovich (2010) found more rumination after material purchases—in balance model terms, material purchases' high balance drives self-referential loops.

4.3.2 Nature Exposure and Neural Mechanisms

Bratman et al. (2015, PNAS) showed that 90-minute nature walks significantly reduced rumination and sgPFC activity versus urban walks. The balance model reading: natural environments

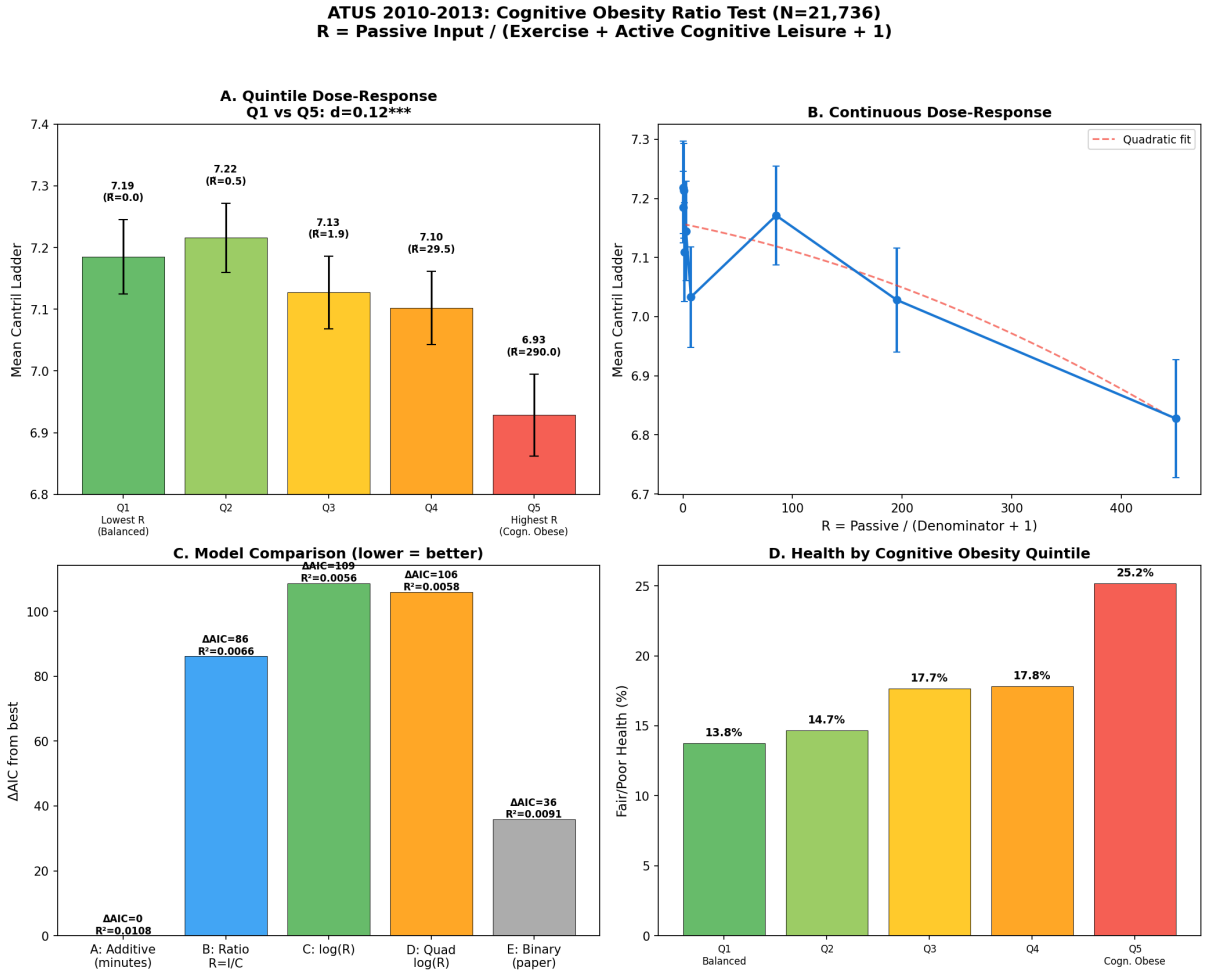

Figure 18: Direct test of the cognitive obesity ratio ($N = 21{,}736$). A: Quintile dose-response of R (Cantril ladder). B: Continuous dose-response curve. C: Model comparison (ΔAIC; additive model best). D: Fair/Poor health by quintile (Q1 = 13.8% $\rightarrow$ Q5 = 25.2%).

minimize Term 1 (low information density) while maximizing Term 2 (physical movement + sensory experience). Urban walks provide equivalent Term 2 but high Term 1 from the urban environment, resulting in insufficient balance improvement. This predicts that "natural" environments should differ in effect depending on information density (quiet forest vs. tourist-crowded park).

4.3.3 Cognitive Starvation as a Prescriptive Concept

If cognitive obesity is chronic elevation of the balance (Term 1 excess), then its intentional reversal can be defined as "cognitive starvation"—deliberate reduction of information input to allow processing capacity to recover. Nature exposure dose-response data provide empirical grounding: Hunter et al. (2019) found a minimum effective dose of 10 minutes; Bettmann et al. (2025, meta-analysis of 78 studies) identified a ceiling effect at 200–300 minutes per week. These thresholds are consistent with a balance mechanism: below 10 minutes, Term 1 reduction is insufficient to shift the balance; beyond 300 minutes/week, Term 2 gains saturate. The concept of cognitive starvation as a prescriptive intervention—analogous to caloric restriction in nutritional obesity—deserves further operationalization.

4.3.4 Exercise Intervention RCT Evidence

Singh et al. (2023, BJSM) meta-analysis of 97 systematic reviews: SMD $= -0.43$ ($k = 218$); umbrella reviews report SMD $= -0.30$ to -0.50. Exercise involves continuous motor output, proprioceptive feedback, and immediate environmental feedback—all closing the sensorimotor loop. Exercise directly increases Term 2 while Term 1 barely changes. Since exercise is randomized, this Term 2 increase causally produces depression improvement.

More recent evidence strengthens this: Fernandez-Montero et al. (2024) reported SMD $= -0.67$ with NNT $= 2.78$, indicating that fewer than 3 patients need to exercise for 1 to achieve clinically meaningful depression reduction. The BMJ network meta-analysis by Noetel et al. (2024) showed that walking, jogging, yoga, and strength training all demonstrated effect sizes comparable to or exceeding antidepressant medication. Critically for this framework, different exercise modalities with varying levels of environmental interaction (yoga > treadmill for loop closure) showed graded effects, though direct comparisons controlling for intensity are lacking.

4.3.5 Self-Efficacy Reinterpreted

Bandura's (1977) self-efficacy—the belief in one's ability to execute actions in specific situations—can be reinterpreted as the subjective perception that one's sensorimotor loop is closed. High self-efficacy: "my actions generate predictable environmental feedback." Low self-efficacy: "my actions do not produce predictable environmental responses"—the subjective experience of an open loop.

Tak et al. (2017) cross-lagged panel model provides an asymmetric causal structure consistent with this reinterpretation: self-efficacy at time t predicts depression at $t + 1$, but depression at

t does not predict self-efficacy at $t + 1$. In balance model terms, loop closure state (proxied by self-efficacy) causally determines balance trajectory, but the balance level alone does not determine loop closure capacity—the relationship is structurally asymmetric.

4.4 Differential Predictions: Balance Model vs. Existing Theories

Table 5: Differential predictions: Balance model vs. existing theories

Prediction	Test condition	ART/SRT prediction	Balance model prediction
1. Nature × smartphone interaction	Nature walk with vs. without concurrent smartphone use	Soft fascination present → effect maintained	Term 1 increase → effect attenuated
2. Indoor exercise vs. nature walk	Intensity-matched comparison of rumination	Equivalent if cortisol reduction equivalent	Indoor = Term 2 only; Nature = Term 1 reduction + Term 2 increase
3. Material purchase × alexithymia	Post-purchase rumination × TAS-20	No prediction	High TAS-20 = low Term 2 → greater balance deterioration
4. Hedonic adaptation speed	Post-purchase happiness decay rate	Explained by memory value	Quantitatively predictable from balance change magnitude

These predictions are not derivable from existing theories and constitute the minimum conditions for the balance model to demonstrate surplus explanatory power. Predictions 3 and 4 connect the framework to behavioral economics: material purchases chronically increase Term 1 while Term 2 barely moves, and alexithymia (measurable via TAS-20) indexes chronic processing compression failure. Section 5 proposes specific experimental designs to test predictions 1–2. Sections 3 and 4 evidence is consistent with these predictions but constitutes exploratory, not confirmatory, support.

5 Operationalization and Proposed Design

5.1 Measuring Experiential Processing: Five Candidate Proxies

This framework requires measurable proxy variables for "experiential processing." Five candidates are identified in order of specificity:

1. **Interoceptive Awareness (MAIA-2)**: The Multidimensional Assessment of Interoceptive Awareness, Version 2 (Mehling et al., 2018) measures awareness of bodily signals across 8 dimensions. MAIA-2 is the most promising individual-level proxy for experiential processing capacity.

2. **Alexithymia (TAS-20)**: The Toronto Alexithymia Scale may index chronic compression failure—inability to convert raw emotional experience into communicable categories.

3. **Experience Sampling Method (ESM)**: ESM can capture minute-level cognitive input vs. experiential processing balance in daily life. The critical target is the momentary balance and its temporal pattern, not total screen time or total physical activity.

4. **ATUS Active/Passive Leisure**: Roy & Orazem (2021) showed active leisure is a "normal good." Our ATUS analysis directly validates this (section 4.2).

5. **Nonlinear Dose-Response**: Meta-analyses report ∼1 hour/day of screen time associated with lowest depression risk. The balance framework predicts this: 1 hour within 10+ waking hours represents low balance; 8 hours represents high balance.

5.2 Proposed Confirmatory Design

5.2.1 Variables

- **Cognitive input**: Screen time (decomposed: SNS, video, news, educational) + emotional media consumption
- **Experiential processing**: Physical activity time + creative activity time + MAIA-2 score (capacity)
- **Balance variable**: Cognitive input − experiential processing
- **Outcomes**: Depression (PHQ-9), alexithymia (TAS-20), wellbeing scale

5.2.2 Predictions

1. The balance variable explains more variance in PHQ-9 than screen time alone (ΔR^2 test).
2. High MAIA-2 scores buffer depression risk at equivalent screen time (MAIA-2 × screen time interaction).
3. SNS (open loop) shows stronger depression association than games (closed loop) after controlling total screen time.
4. Alexithymia emerges as consequence rather than cause of chronic balance imbalance (mediation analysis).

5.2.3 Design

ESM study, $n = 50$–100, 14 days, 8 prompts/day. Wearable integration for automatic physical activity detection. Pre/post: PHQ-9, TAS-20, MAIA-2. Primary analysis: multilevel model with within-person balance as time-varying predictor. Power: Level 2 = 50× Level 1 = 112 observations (8/day × 14 days) = 5,600 observations achieves 80% power for multilevel models (per Arend & Schäfer, 2019).

5.3 Minimum-Cost Verification Designs

Experiment A (Nature × Smartphone Interaction): Extend Bratman et al. (2015) 2-group RCT with a 3rd group "nature walk + free smartphone use." $N = 60$ (20/group). Balance model: nature > nature+smartphone > urban. ART: nature ≈ nature+smartphone > urban. Cost: ∼1/10 of Bratman et al. (no fMRI needed).

Experiment B (Indoor Exercise vs. Nature Walk): 2 × 2 between-subjects: exercise condition (indoor treadmill vs. nature walk) × time. Intensity matched via wearable. Balance model: nature walk > indoor exercise (Term 2 equivalent but Term 1 differs). SRT: equivalent if cortisol reduction equivalent. $N = 50$.

Both are implementable at undergraduate thesis scale (∼110 participants, ∼3 months).

6 Falsification Conditions

The following results would falsify the core claims of this framework:

1. The balance variable (cognitive input − experiential processing) does not explain additional variance in depression beyond absolute screen time at the individual level.
2. After controlling sensorimotor loop closure, games and SNS show equivalent depression risk.
3. No MAIA-2 × screen time interaction for depression (interoceptive capacity does not buffer).
4. The internet/education ratio is a weaker macro-level depression predictor than internet penetration alone.
5. The correlation structure shift (high-income vs. low-income countries) disappears when controlling healthcare access.
6. Structured therapeutic AI (closed-loop design) and general-purpose AI (self-referential loop permitting) show equivalent effects on depression symptoms.

6.1 Honest Assessment of Current Evidence

7 Discussion

7.1 Positioning Relative to Existing Literature

This framework occupies a currently vacant niche. Haidt (2024) and Twenge (2017) focus on individual-level screen time effects without international epidemiology or balance-based mechanisms. Achenbach's internalizing/externalizing classification does not stratify by development stage. The displacement hypothesis examines whether screen time displaces physical activity

Table 6: Honest assessment of current evidence

What this paper provides	What this paper does not provide
A new organizing axis (balance) that reorganizes existing confusion	Individual-level confirmatory causal evidence
Macro-level exploratory evidence consistent with predictions (177 countries)	High explanatory power ($R^2 = 0.07$–0.13)
Integration of three previously unconnected literatures (information overload, exercise, digital media)	Direct operationalization of "loop closure degree"
Specific falsification conditions	Resolution of ecological fallacy
Individual-level cross-sectional support (NHANES $N = 5{,}032$; ATUS $N = 21{,}736$) with additive model confirmed (ΔAIC ≥ 36)	Individual-level proof that balance > absolute quantity
Verification roadmap with implementable experiments	Prospective longitudinal or natural experiment identification

but not balance or processing modes. Whitworth (2009) proposed a nutritional metaphor without quantitative data. This framework integrates these through (a) proposing balance as the primary variable, (b) providing 177-country macro-level evidence, (c) connecting to processing mode theory, and (d) deriving a threshold model explaining development-stage-dependent correlation structures.

The relationship with Behavioral Activation (BA) therapy is particularly important. BA is among the most evidence-based psychotherapies for depression (Dimidjian et al., 2006). BA's "shift from rumination to direct experience" is substantively identical to this model's "shift from self-referential loop to closed loop." BA's clinical success serves as indirect corroboration of the balance mechanism. This model adds loop closure as a structural dimension: SNS interaction rated pleasure 8/10 in BA would still be predicted to have limited balance improvement if it is open-loop.

7.2 Policy Implications

If this framework is correct, current policy debate is structurally incomplete. SNS age restrictions (regulatory camp) and digital literacy education (literacy camp) address only the input side of balance. Term 2—experiential processing capacity—requires entirely different policies: mandatory physical education time, outdoor activity time, arts and crafts curricula.

Core policy insight: SNS age restrictions and physical education hours should be treated as a **single policy package**, not alternatives. Reducing Term 1 without increasing Term 2 (or vice versa) is insufficient.

Advertising format regulation constitutes a third policy axis. The loop closure threshold of this framework suggests that non-skippable autoplay ads are more harmful than user-selected content, supporting policy design targeting "non-selective cognitive input density" rather than mere screen time.

Natural experiment: Japan's Work Style Reform. Japan's 2018 Work Style Reform Act mandated enforceable overtime caps, interpretable as a policy reduction of involuntary cognitive input. Japan's suicide rate declined from a mean of 20.70 (2010–2017) to 16.98 (2018–2021), an ≈17.9% reduction that substantially exceeds the OECD 8-country average change (−0.2%; DID estimate = −17.7 percentage points). However, Japan's suicide rate had been declining consistently since a 2003 peak (pre-trend = −0.39/year), making it difficult to disentangle reform effects from pre-existing trends with current data. Causal identification requires more refined analysis exploiting the reform's staggered implementation.

Ethical caveats. This framework should *not* be used as a justification for restricting individuals' access to information. The policy target is the structural incentives of platforms (advertising density, autoplay, notification design) rather than individual consumption choices. Any application of this framework must guard against two misuses: (1) the individual blame model, which attributes systemic problems to personal "information hygiene" failures; and (2) authoritarian instrumentalization, which uses "cognitive protection" as a pretext for censorship. The framework's policy implications are structural (platform design regulation, mandatory physical education, advertising format restrictions) rather than individual-prescriptive.

7.3 AI Design Implications: The Principle of Cognitive Motorization

If this framework is correct, AI systems can prevent cognitive obesity by detecting open-loop or self-referential loop patterns and converting them to closed-loop engagement—a process we term "cognitive motorization." For example, when a user engages in rumination-like repetitive querying, the AI could introduce Socratic questioning, behavioral activation prompts, or structured problem-solving frameworks that generate genuine prediction errors and require agentic responses.

Ethical limits of cognitive motorization. Three constraints apply: (1) for patients with severe depression, AI-initiated behavioral change suggestions carry clinical risk and should defer to professional intervention; (2) loop detection accuracy is insufficient to warrant autonomous intervention—false positives (misidentifying productive deep thinking as rumination) could be counterproductive; (3) commercial AI systems face a structural incentive to revert to sycophancy (which maximizes user engagement metrics), undermining any therapeutic loop-closure design. These constraints suggest that cognitive motorization is more appropriate as a design principle for dedicated therapeutic AI than as a feature of general-purpose conversational AI.

7.4 Ethical Considerations: Equity Implications

The service sector employment moderation effect identified in section 3 (interaction $F = 73.5$) highlights an equity dimension: individuals in cognitively demanding service occupations face amplified depression effects from digital saturation. This "cognitive protection inequality" parallels occupational health disparities: knowledge workers and gig economy participants may have systematically higher cognitive load baselines, reducing their capacity to benefit from the same

Term 2 interventions that help others. Policy responses should include occupational-specific cognitive load assessment and targeted access to restorative environments.

7.5 The Observation Problem

A methodologically important implication: the same external behavior may constitute cognitive input for one individual and experiential processing for another. A programmer debugging code (closed loop) and a person scrolling social media on the same screen are at opposite ends of the balance spectrum. This observation problem means that external behavioral measures (screen time, app category) are fundamentally insufficient for testing this framework. Individual-level assessment of loop closure state—through ESM or wearable-based interaction contingency detection—is required.

7.6 Relationship with "Cognitive Starvation"

If cognitive obesity is a state where "Input $\gg$ Processing," its mirror image "Cognitive Starvation" (Processing $\gg$ Input) should also exist. Candidates include: intellectually understimulating environments (certain institutional settings, monotonous work), sensory deprivation, and information-poor environments. This generates a novel prediction: cognitive starvation and cognitive obesity should converge on similar outcomes (depression, anxiety) via different pathways, but starvation should be accompanied by boredom and stimulus-seeking, while obesity should be accompanied by overwhelm and avoidance. This prediction is testable via ESM.

Ethics Statement

This study is based entirely on publicly available, de-identified secondary datasets: NHANES (NCHS/CDC), ATUS Wellbeing Module (BLS), WHO Global Health Observatory, World Bank World Development Indicators, IHME Global Burden of Disease, and UNODC homicide statistics. No primary data collection involving human participants was conducted. Under 45 CFR §46.104(d)(4) (U.S. Common Rule), research involving the analysis of publicly available, de-identified data does not constitute human subjects research and is exempt from Institutional Review Board (IRB) review. As the author is an independent researcher without institutional affiliation, no institutional ethics board holds jurisdiction; the exemption criteria are satisfied by the nature of the data sources themselves.

Acknowledgments

Author's Premises

This paper was written by a non-specialist who is not a professional researcher. This is not modesty but a structural constraint relevant to interpreting the work.

Advantages of this position: (1) the freedom to propose cross-domain integration that field specialists find difficult; (2) the motivation to construct a testable framework rather than defend

a particular paradigm; (3) the lack of career incentives tied to specific conclusions.

Disadvantages: (1) blind spots in domain-specific methodological nuances; (2) possible insufficient grasp of unpublished knowledge; (3) limited peer network for early-stage feedback. These limitations are partially mitigated by the open-source replication repository that allows method-level verification.

A note on AI assistance: This paper was written with extensive assistance from large language models (Claude, GPT). The role of AI was: (1) literature search and organization, (2) statistical analysis code generation and verification, (3) English translation assistance, (4) logical consistency checking. All substantive claims, interpretive directions, and methodological choices are the sole responsibility of the author.

This paper is a working paper and welcomes all forms of feedback: methodological corrections, overlooked literature, alternative interpretations, and collaboration proposals.

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Data Availability and Replication Materials

All analysis scripts (15 Python + 1 R), data download procedures, alternative estimators (Driscoll-Kraay SE, Pesaran CCE, Bai IFE, first-difference IV), survey-weighted and covariate-adjusted variants, ad proxy validation data, BibTeX references, publication-ready figures, and paper-to-code correspondence tables are available at:

https://github.com/miyauchikazuyoshi/cognitive-obesity-replication

A Data Specifications

A.1 NHANES 2017–2018 Specifications

Data source: CDC NHANES 2017–2018 cycle

Files used: DEMO_J.XPT, DPQ_J.XPT, PAQ_J.XPT, BMX_J.XPT

Sample: Adults 18+ with complete PHQ-9 responses ($N = 5{,}032$)

Outcome: PHQ-9 sum score (0–27; clinical depression ≥ 10)

Key predictor: Leisure physical exercise (binary: "vigorous/moderate recreational activity" in past 30 days)

Covariates: Age, sex, education (5-level), poverty-income ratio, health insurance status, BMI

Exclusion: Missing PHQ-9, age < 18

A.2 ATUS Wellbeing Module Specifications

Data source: Bureau of Labor Statistics, American Time Use Survey Wellbeing Module (2010, 2012, 2013)

Sample: Adults 15+ with complete Cantril ladder responses ($N = 21{,}736$)

Outcome: Cantril life satisfaction ladder (0–10)

Activity classification: Passive leisure (T120303+T120306), Active cognitive leisure (T120101, T1202xx, T120307–T120313, T1501xx, T150201), Physical exercise (T130101–T130199, T130232)

Covariates: Age, sex, education, household income